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February 23, 2026

Robert F. Kennedy, Jr.,
Secretary, Department of Health and Human Services
200 Independence Ave, SW
Washington, DC 20201

Submitted electronically via Regulations.gov

RE: RIN 0955–AA13: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Secretary Kennedy:

Dexcom, Inc. is submitting this letter in response to the Request for Information published by the Department of Health and Human Services on December 23, 2025.[1]

Founded in 1999, Dexcom, Inc. is the market leader in transforming diabetes care and management by providing superior continuous glucose monitoring (CGM) technology to help patients and healthcare professionals better manage diabetes. Since our inception, we have focused on better outcomes for patients, caregivers, and clinicians by delivering solutions that are best in class—while empowering our community to take control of diabetes. In 2017, we obtained first-time Medicare coverage for a CGM system, and we continue to innovate with the G7 CGM system. One of our latest innovations, the Smart Food Logging feature of our app, permits users to take a photo of their food and receive AI-powered help to identify the ingredients and populate a meal description. This makes it easier for users to correlate meals with specific glucose movements determined by their CGM and better understand the impact of their dietary choices.

We appreciate the opportunity to respond to this RFI.

Background on Continuous Glucose Monitoring (CGM)

A CGM is a small device, about the size of two stacked quarter, that adheres to the skin and uses a very fine wire probe, placed into the interstitial area of the skin, to constantly measure glucose

[1] 90 Fed. Reg. 60108 (December 23, 2025).

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levels in the body. The data thus measured is reported every five minutes to a dedicated "receiver" or to the user's smart device.

Individuals who use a CGM can immediately see how their dietary, activity and medication choices impact glucose levels. This empowers them to take charge of their own condition immediately, rather than waiting until they can receive help from a provider. The data can also be shared with family and caregivers who can also encourage healthy choices and customize plans of care to the individual.

Use of CGM to manage diabetes is so well established that the American Diabetes Association's Standards of Care in Diabetes recommend CGM use for both people using insulin and those who do not use insulin.[2] This standard of care is backed by a vast body of clinical literature.

Headers below reflect specific questions raised by HHS in the RFI.

What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

AI in clinical care presents distinct privacy, security, and data-governance challenges. These include uncertainty around lawful secondary use of health data for AI training and improvement, application of purpose limitation principles in machine-learning contexts, data provenance and representativeness, and emerging cybersecurity risks such as model exfiltration or inference attacks. At the same time, expectations around transparency and explainability continue to develop, particularly given the technical opacity of many high-performing models and the differing informational needs of clinicians, patients, and regulators.

AI systems also distribute decision-making across developers, healthcare organizations, clinicians, and data providers, creating uncertainty around liability, indemnification, and professional responsibility – particularly when models are updated or retrained over time. Existing regulatory models often assume fixed functionality and discrete approval events, while many AI systems evolve continuously, raising unresolved questions about when changes trigger revalidation, how performance drift should be monitored, and how longitudinal accountability should be documented.

[2] American Diabetes Association Professional Practice Committee for Diabetes; 7. Diabetes Technology: Standards of Care in Diabetes—2026. *Diabetes Care* 1 January 2026; 49 (Supplement_1): S150–S165. https://doi.org/10.2337/dc26-S007

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The U.S. Department of Health and Human Services is well positioned to help address these challenges by clarifying important aspects related to legal implementation roles and responsibilities across the AI value chain:

- HHS could play a coordinating role by aligning expectations across its privacy, security, quality, and safety functions with clearer guidance on lawful secondary use of health data for AI, including acceptable de-identification and privacy-enhancing techniques. This could include discussion of the use of identifiable data in the development of AI models. For example, whether training less advanced models with identifiable data in certain limited circumstances would be appropriate.

- HHS could issue guidance articulating shared but differentiated expectations for developers, healthcare organizations, and clinicians, helping to align legal accountability with operational control without displacing existing standards of care. Such clarity would reduce uncertainty in contracting, indemnification, insurance coverage, and clinical risk management.

- HHS could further support responsible adoption by promoting lifecycle-based governance models that emphasize continuous monitoring, tiered change management, and post-deployment oversight. In addition, HHS could reduce burden and inconsistency by publishing standardized documentation templates for AI risk assessments, validation summaries, monitoring records, and incident reporting.

Taken together, actions like these would improve patient safety and trust while making healthcare AI governance more predictable, proportionate, and aligned with clinical and technical realities.

What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

Many medical devices today, including CGMs, are used in conjunction with an app. Like Dexcom, device manufacturers are increasingly looking to include AI-powered features in their apps. Development and testing of such features require significant, ongoing expenditure of resources. Further, as they become more powerful, these tools add increasing value to the device

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itself. Given how critical these digital aspects are becoming, it is reasonable to see them as a "supply" that is necessary to the full functionality of the device.

A good example of how digital elements of a device system have become indispensable is seen in the "automated insulin delivery" (AID) systems currently on the market. An AID system is a three-part tool consisting of an insulin pump, a CGM and an algorithm. In these systems, the CGM provides data on glucose levels, the pump delivers insulin and the algorithm stands between them, taking data from the CGM and dictating to the pump how much insulin is to be delivered. These algorithms have been independently recognized as devices by the FDA and are distinguished from the pumps and CGMs as such. Each of the pump manufacturers operating in the US provides an algorithm with their device and there are also two other organizations that do not manufacture any physical device, but independently produce an algorithm for automated insulin delivery that has been FDA approved. These algorithms are indispensable for the operation of an automated insulin delivery system and their maintenance and continued updating require ongoing investment.

The definition of "durable medical equipment" in the Social Security Act at Section 1861(n) and in the regulation at 42 CFR 414.202 do not specify that either durable medical equipment (DME) or the associated supplies must be tangible, physical items. Increasingly, digital elements of a device, including AI-enabled tools and algorithms, are being recognized by the FDA as a category of "device" themselves and are producing more meaningful clinical impacts.

Dexcom recommends HHS consider clarifying the regulation at 42 CFR 414.202 such that it recognizes that when a digital device is independently recognized by the FDA, it can qualify as either a device or a supply and therefore be eligible for potential reimbursement under the Medicare program.

We thank you for the opportunity to provide input in response to this RFI. Should you have any questions about these comments, please contact Jesse Bushman at jesse.bushman@dexcom.com.

Sincerely,

/JSB/

Jesse S. Bushman
Director, US Policy
Dexcom, Inc.